

NURS 280 — Human Sexual Health · Midterm Cram Sheet

UBC · Instr. Kristen Gilbert · Exam: Tue June 2, 1–3pm. High-yield only — pair with the study tool for depth.

L1 · INTRO TO HUMAN SEXUALITY

WHO sexual health (2006)

Physical, emotional, mental & social well-being re: sexuality — **not merely absence of disease/dysfunction**. Needs a positive, respectful approach; possibility of **pleasurable, safe** experiences free of coercion, discrimination, violence; sexual **rights** respected, protected, fulfilled.

Dailey's 5 Circles of Sexuality (1981)

- **Sensuality** — senses/pleasure: skin hunger, body image, response cycle, fantasy.
- **Intimacy** — emotional closeness: caring, trust, vulnerability, self-disclosure.
- **Sexual Identity** — biological sex, gender identity, gender expression, gender role, orientation.
- **Sexual Health & Reproduction** — anatomy, intercourse, contraception, abortion, STIs.
- **Sexualization** — the **power circle**: using sex to influence/control (flirting, media, harassment, assault).

L2 · THEORY & RESEARCH

- **Stayton** — 3 reasons sex is hard to discuss: **Ignorance, Secrecy, Trauma**.
- **Alfred Kinsey** — first scientific study of sex in N. America, **1940s**; **Kinsey Scale 0–6** (hetero→homo). (cf. Klein Grid; van Anders' Sexual Configurations Theory.)
- **Masters & Johnson** — began **1950s**; direct **lab observation** of physiological response → the 4-phase cycle.

L3 · SEX EDUCATION IN CANADA

- Education is a **provincial/territorial** responsibility → wide variation. **BC curriculum since 1995** (mandated, delivery 'hit or miss'). **Gatekeepers**: school boards, principals, parents/community, teachers.
- **SIECCAN (2018) 9 principles** of comprehensive sex ed: accessible to all; human rights; scientifically accurate; broad scope; LGBTQI2S+/asexual inclusive; gender equality & violence prevention; balanced (positives + prevention); responsive to emerging issues; skilled educators.
- **USA AOUM** (abstinence-only-until-marriage) = scientifically/ethically problematic, **ineffective** at delaying sex, withholds info (J. Adolescent Health 2017). **Netherlands** = best outcomes.

L4 · VALUES & BELIEFS

- Sources of values: **family, formal/informal education, peers, media, faith, personal experience**.
- **Schalet (2004) 'sleepover' study**: ~9/10 US parents **forbid** a teen's partner overnight (dramatization); ~9/10 Dutch parents **permit** it (normalization). UNESCO (2009): comprehensive ed lets youth explore values & practice decision-making.

Marker	US	Can	Neth
Teen pregnancy*	57	28	21
Age of sexual debut	16	17	17
3+ partners in HS	19%	—	4%
'Love' = reason 1st sex	59%	—	73%

*per 1,000 females 15–19. Normalization → lower teen pregnancy, later/safer debut.

L5 · ANATOMY & PHYSIOLOGY

- Structures are **homologous** (clitoris↔penis, labia↔scrotum, ovaries↔testes). **Sex is not strictly binary**; intersex variation is natural ('Sex redefined').
- **SPERM path**: Seminiferous tubules → **epididymis** (mature/store) → vas deferens → ejaculatory duct (+ seminal vesicle fructose, prostate alkaline, Cowper's pre-ejaculate) → urethra → out.
- **EGG path**: Ovary (ovulation) → fimbriae → **fallopian tube = site of fertilization (high in tube)** → uterus (implant) or shed as menstruation.
- **Clitoris** largely internal, only known function = **pleasure**. 'Hymen' → more accurately **vaginal corona**; NOT a reliable virginity indicator.

L6 · AROUSAL PATTERNS

Masters & Johnson — 4 phases

- **Excitement**: HR/BP/breathing up; **vasocongestion** & erection (clitoris/penis/nipples); testicles draw up; vaginal lubrication (white blood cells through the wall). Often passive/automatic.
- **Plateau**: pleasure peak before orgasm; 'sex flush'; pre-ejaculate (Cowper's gland is **IN THE BODY**, not the penis); clitoris very sensitive; vagina lengthens = **tenting**.
- **Orgasm**: involuntary rhythmic contractions; BP/HR/breathing peak; release of tension.
- **Resolution**: return to baseline; penis has a **refractory period** (lengthens with age); **clitoris has none** (possible multiple orgasms).
- **Ejaculation ≠ orgasm** (separable). Only ~20% orgasm from intercourse alone; most clitorises need direct stimulation. 'Blue balls' = NOT a medical condition (manipulation myth).

Other models

- **Kaplan (1974)**: Desire → Excitement → Orgasm (added 'desire').

- **Basson (circular/nonlinear)**: female desire often **responsive, not spontaneous**; intimacy → seek stimuli → arousal → responsive desire.
- **Dual Control (Bancroft & Janssen)**: balance of **SES (accelerator)** vs **SIS (brakes)**; popularized by Nagoski (turn on the ons, turn off the offs).
- **Nagoski — arousal nonconcordance**: genital response predicts wanting only 10–50%; physical arousal ≠ desire/consent.

L7 · CONCEPTION, PREGNANCY & BIRTH

- Conception **high in fallopian tube, mid-cycle**. Egg viable ~**24h**; sperm live **5–7 days**; ~1,000 sperm reach egg and the **egg selects** the sperm. **Implantation ~2 wks** later = 'pregnant.'
- **40–50%** of Canadian pregnancies unplanned. ~**15%** couples face infertility (test after 1 yr; BC covers 1 IVF round; **AB/SK cover none**). **Miscarriage** common (~1/3, first 7 wks, usually genetic). **Ectopic** = outside uterus (usually tube) = **100% emergency**.
- Gestation **280 d / 40 wk** (counted from last period). **Term = 37 wk**; **post-dates = 42 wk**. Trimesters: 1st = organs form (**critical period of vulnerability**); 2nd = viability/growth; 3rd = head down.
- Labour: **Stage 1** cervix dilates/effaces (to ~10 cm); **Stage 2** baby born; **Stage 3** placenta. **Appar at 1 & 5 min**. BC: OB/family MD, midwife, ± doula.

L8 · CONTRACEPTION

- **4 mechanisms**: Hormonal (**stop ovulation**), Barrier (block sperm), 'Natural' (rules of conception), Emergency (interrupt conception).
- **Hormonal all work the same**: estrogen+progesterone keep hormones **above the ovulation threshold** → no egg. Pill ~99% perfect / 93–94% typical use; also Patch (Evra), Ring (NuvaRing), Injection (Depo), Implant (Nexplanon).
- **Barrier**: external/internal condom, IUDs. **'Natural'**: abstinence, withdrawal, sympto-thermal (BBT + cervical fluid + cervix; ~75%), lactational amenorrhea (98% if <6 mo + no period + nursing on demand).
- **Prescription contraception is FREE in BC** (AccessBC). Pharmacists can prescribe.

Emergency contraception

- **Plan B (levonorgestrel)**: **delays ovulation** only (Schedule 3, OTC, free in BC); useless if already ovulated; not an abortion; less effective ≥165 lb. ~5 days.
- **ella (ulipristal)**: works later/better, wider weight range — **but prescription**.
- **Copper IUD = most effective EC** (within 7 days; **foreign-body effect**). IUDs are classed as **barrier** (even hormonal IUD = thickens cervical fluid plug).

L9 · PREGNANCY OPTIONS

- **3 options**: parent · adoption · abortion.
- **Adoption**: Confidential / Open / Direct / Custom (Indigenous). Consent from **both birth parents, any age**, given **10 days** after birth, withdrawable within **30 days**. Records open at 19 (since the 1996 disclosure ruling).
- **Abortion law**: illegal pre-1969 → **1969 Criminal Law Amendment Act** legal only if a **Therapeutic Abortion Committee (TAC)** approved → **R. v. Morgentaler (1988)** struck it down (violated **security of the person, Charter s.7**). **No abortion law since**.
- **Procedural** (suction, <10 min, very safe) vs **Medication** = Mifegymiso (mifepristone + misoprostol), **<10 wk** (covered everywhere except Nunavut). **87%** in first 12 wk; ~1/3 of people who can get pregnant have one; ~1/3 already parents.
- **Repro justice**: birth alerts (now illegal in BC; 'Baby M'), 60s Scoop, forced sterilization — disproportionately Indigenous (6% of kids, ~50% of foster care).

L10 · STIs — THE 'BIG EIGHT'

- **Bacterial (curable w/ antibiotics)**: Chlamydia · Gonorrhea · Syphilis · Pubic lice* (*parasite).
- **Viral (manageable, not curable)**: Herpes · Hepatitis B · HIV · HPV. (HPV/Gardasil-9 & Hep B are vaccine-preventable.)
- **Transmission**: fluids (blood, semen, vaginal/rectal fluid, breast milk) + skin-to-skin (genitals, mouth, anus).
- **Testing (3)**: visual check · swab/urine · blood. (BC: Student Health, Options, BCCDC, GetCheckedOnline.)
- **Prevention**: barriers (condoms/dams) · vaccination · **PrEP** (pre-exposure prophylaxis for HIV; U=U) · regular testing · partner notification.

SUPP · BASSON (2001), FEMALE SEXUAL RESPONSE

- Women's desire is largely **responsive, not spontaneous**; motivated by **emotional intimacy**. The linear Masters/Johnson/Kaplan cycle is insufficient for many. Arousal is **biopsychosocial** — correlates with how mentally exciting the stimulus is, poorly with genital blood flow. Drugs (e.g., sildenafil, testosterone) usually **insufficient alone** unless psychological factors are addressed.